More Than a Chart: Why the Small Things About Your Wife Belong in the Care Conversation

A short guide to what new caregivers can use about her ordinary day, and how it differs from her care orders.

The new team will get your wife's chart: the diagnoses, the medications, the forms. Nothing on it says she takes her tea before anything else, or that she can't stand being called “sweetie.”

This guide explains why details like those belong in the care conversation, how they differ from the instructions her clinicians have given, and what a family can do with them. It explains. It doesn't write her page for you.

Introducing her is part of the handoff

The new team will learn your wife's medical details from her records. Who she is has to come from her, and from you. The Family Caregiver Alliance's advice for a move into residential care is to introduce your loved one, and yourself, to the staff and tell them about her life. Writing about dementia care at home, the Alzheimer's Association puts it this way: the more care providers know about the person they are caring for, the better care they can give. [1] [2]

In a nursing home it is also written into the rules. Medicare's guide for residents says nursing homes must recognize and respect a resident's preferences when giving care, and must develop care plans that describe how they'll meet her needs and preferences. That guide covers nursing homes only. If she's going somewhere else, or care is coming to the house, ask the provider how preferences are recorded there. [3]

- Choose the details someone could use on the first day: her name, her morning, her meals, what settles her.
- Keep it to one page. A new caregiver has about two minutes for it.
- Ask whoever receives it to confirm they have it.

Her ordinary day, in her words

You could describe her day yourself, and you'd get most of it right. Ask her anyway. The details she'd put first may not be the ones you would.

A workbook from The Conversation Project and Ariadne Labs offers a simple question: “What does a good day look like for you?” Its examples are everyday ones: get up and dressed, play with my cat, make a phone call, have coffee with a friend. For anyone helping someone else answer, it says to take it in small pieces, that it's always okay to skip a question, and to take a break if the person gets tired. [4]

For the kind of detail worth catching, look at the headings of a form the Alzheimer's Association gives families, called Personal Facts and Insights: a preferred name, a typical day, established routines “such as having coffee and newspaper in the morning,” what is calming, and what is irritating or upsetting. [5]

- Write her words down as she says them. “Tea first, then I can face people” is a better line than “prefers tea.”
- Include the familiar people as well as the familiar routines: who calls, who visits, whose photograph she keeps close.

A preference is not an instruction

Two kinds of statement end up in the same notes. “She doesn't like fish” is her preference. “She has a nut allergy” is something a clinician told you. Both are about food, and if they sit in one list a busy reader may treat both as optional, or both as orders.

MedlinePlus, from the National Library of Medicine, describes a food allergy as an abnormal immune system reaction to certain foods. That is a medical matter, not a like or a dislike. The Alzheimer's Association's form shows one way to keep things apart: special dietary needs, food allergies, favorite foods and strong dislikes each get their own line. [6] [5]

Do the same in your notes. Her preferences go on one list, and anything a doctor, nurse or dietitian has given you goes on another, with who said it and when. If you can't tell which list something belongs on, ask the care team which it is.

Preferences are also where the family's view and hers can part. The National Consumer Voice tells families of nursing home residents that their role is to communicate their loved one's needs and preferences, and that this “may be different than what you think is best for your loved one.” When you notice a difference, write down both and mark which is hers. [7]

- For each thing she enjoys, note what help she'd need to take part: a ride, her glasses, someone to walk with. Writing it down doesn't sign her up. Taking part stays her choice.

From notes to a page she has seen

The roadmap calls the finished page an About Me profile. Before any version of it is handed over, show it to her or read it to her, where she's able. Ask what she wants shared and what she'd sooner keep to herself.

Then expect it to change. The Conversation Project's workbook notes that people's preferences often change as their health changes or as time goes by, and suggests revisiting the answers. For nursing homes, Medicare's guide says much the same: preferences and goals may change during a stay, so the home should have ongoing discussions with the resident about updating the care plan. Put a date on the page, and the name of whoever wrote it. [4] [3]

What this means for your family

- The chart tells the new team what your wife has. The page tells them who she is, and it comes from her and from you.
- Her words go first. Yours are marked as yours, and anything she hasn't confirmed is marked too.
- Preferences and clinician-given instructions go on separate lists. A note you can't place goes to the care team as a question.
- A page is finished when someone on the new team has it and you know who.
- How her routines fit around her care in the new setting is for the people who care for her to work out with her. Your page gives them something to start from.

What this guide doesn't do

It doesn't explain any medical condition, say which of her routines can be kept, or decide what she would want if she can't say. The quotations and the points about nursing homes come from the sources below. Some of those pages were written for families living with dementia; we've used only what they say about getting to know a person, and nothing here assumes anything about your wife's health. The About Me name and the advice about what to write down are ours.

Ready for the next step?

You can now say why her routines belong in the handoff, and how they differ from her care orders. The care interview asks about each part of her day in turn, alongside her care needs and the support around her, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Family Caregiver Alliance, Residential Care Options: Caregiving Doesn't End When Your Loved One Moves
2. Alzheimer's Association, In-Home Care
3. Centers for Medicare & Medicaid Services, Your guide to living in a nursing home (CMS Product No. 12217)
4. The Conversation Project (Institute for Healthcare Improvement) and Ariadne Labs, What Matters to Me: A Workbook for Better Health Care
5. Alzheimer's Association, Personal Facts and Insights (form)
6. MedlinePlus (National Library of Medicine), Food Allergy
7. National Consumer Voice for Quality Long-Term Care, Family Involvement in Nursing Homes (fact sheet)

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